

PharmX Technologies Limited PHX

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by

Ares

			2026	2027
Price:	0.09	EPS	0	0
Shares Out. (in M):	693	P/E	0	0
Market Cap (in \$M):	62	P/FCF	0	0
Net Debt (in \$M):	-2	EBIT	0	0
TEV (in \$M):	60	TEV/EBIT	0	0

Description

I. PharmX Is the Critical Infrastructure for Australian and New Zealand Pharmacy, and Its Shares Are Available at a Deeply Mispriced Valuation

PharmX is the dominant electronic ordering backbone for the Australian and New Zealand pharmacy industry that is going through business model and revenue model transitions. If these transitions are successful, PharmX could become a multi-bagger investment.

Roughly 7,000 pharmacies, which is about 99% of the entire industry, order goods from suppliers through PharmX's infrastructure, and PharmX works with the vast majority of the major wholesalers, suppliers, and distributors in the region. This is about as close to ubiquitous as a piece of B2B infrastructure gets.

Here is the problem, and it is also the opportunity: in fiscal year 2026 (ended June 30, 2026), approximately AUD 26 billion of gross merchandise volume (GMV) flowed across PharmX's platform, and PharmX kept AUD 7.7 million of revenue for itself. That is an effective monetization rate of about 3 basis points.

Why would a company that touches almost everybody in the industry and in theory has this much leverage over an entire industry charge so little? Because PharmX was not built to maximize profit. It was founded in 2006 by a group of point-of-sale (PoS) software companies as shared industry infrastructure, priced to cover costs rather than to capture value. PharmX's story closely rhymes with the early history of Mastercard. For most of its life, PharmX charged suppliers a flat monthly fee per pharmacy connection (approximately AUD 17 per month), regardless of how much GMV moved across that connection. A supplier moving AUD 1,000 through PharmX paid the same monthly fee as a supplier moving AUD 1,000,000.

That underpricing is now being corrected, and the correction is the entire thesis. PharmX is in the early stages of converting itself from a fixed-fee connectivity utility into a take-rate marketplace, where PharmX's take rate is a percentage of GMV that changes hands across its platform rather than a flat toll per connection. Management has laid out where those take rates land: roughly 0.5% to 5% depending on the supplier and the type of product, versus the roughly 3 basis points the company earns today. Even a modest, partial migration of GMV onto this new pricing structure would multiply PharmX's revenue several times over, and because the infrastructure is already built and already in use by essentially the entire industry, very little incremental capital is required to get there. Importantly, due to its legacy PharmX does not face a cold start problem that is the foundational challenge for any network effect business.

The stock does not reflect this. PharmX trades today at approximately AUD 0.09 per share, for a market capitalization of about AUD 68 million and an enterprise value of about AUD 62.5 million. That is roughly 7.8x 2026 revenue (ended June 30, 2026) and roughly 55x 2026 EBITDA, so this is not a statistically cheap stock on a backward-looking basis. However, the backward-looking numbers still reflect a business earning 3 basis points on its GMV (e.g., no credit for likely revenue growth) while this business already spent money to build a marketplace (e.g., elevated costs) and shows early signs of traction.

What makes PharmX interesting is the size of the gap between what the business earns today and what it could plausibly earn once even a modest share of that AUD 26 billion of GMV is repriced. Even if half of the AUD 26 billion GMV migrates to a take rate model (e.g., AUD 13 billion) and even if PharmX gets 1% take rate, then revenue would be AUD 260 million (vs. AUD 7.7 million in 2026). If PharmX gets to a 50% EBITDA margin (not unreasonable for a marketplace), then EBITDA could reach AUD 130 million in several years. With the market cap of less than AUD 65 million as of right now (and effectively no debt) ... well, it could be a successful investment.

II. What PharmX Actually Does, in Plain English

PharmX operates three connected pieces of infrastructure for the pharmacy industry in Australia and New Zealand.

The core, and by far the most established, is an EDI (electronic data interchange) Gateway. Think of it as the plumbing that lets a pharmacy's point-of-sale system talk electronically to a supplier's ordering system. As a result, an order, an invoice, and a confirmation can move back and forth without a human re-keying anything. If you know SPS Commerce in the US (SPSC), you can use it as an analogy, except PharmX focuses on ANZ and only on the pharmacy industry. It is not a glamorous business, but it is the kind of business that becomes very hard to dislodge once it is embedded in thousands of workflows: pharmacists could easily spend up to 1-2 hours a day ordering goods from multiple suppliers. PharmX is the rail that most of that traffic runs on, and it has been for close to two decades.

The newer, and much more important piece for this thesis, is the Marketplace: a true e-commerce platform layered on top of the same underlying network, where pharmacies can browse, compare pricing and availability, and order across many suppliers in a single checkout. This is the way a business would shop on a well-run B2B version of Amazon. Since the Marketplace is fully integrated with the EDI Gateway, PharmX already knows a given pharmacy's historical purchasing pattern, which lets the platform pre-build a pharmacy's typical reorder basket rather than starting from a blank cart.

The third, and still emerging, piece is a Data and Analytics layer. Two decades of transaction history across nearly the entire industry is a genuinely unusual dataset, and PharmX is beginning to sell insight products, drawn from that data, to suppliers and wholesalers who want to understand what is selling, where, and to whom.

The unifying idea across all three pieces: PharmX does not need to go build the network; it already has one. What it needs to do is finish converting how it gets paid for the network it has already built.

III. The Founding Story Explains the Chronic Under-Monetization

Let's discuss why PharmX ended up so badly under-monetized, because understanding the "why" is what convinced me this is fixable rather than a permanent structural feature of the business.

PharmX was established in 2006 by four Australian pharmacy point-of-sale companies as a joint venture, built to give the industry shared connectivity infrastructure rather than to maximize the profits of any single owner. I will skip a lengthy corporate history that had M&A and divestitures. What is important is that today PharmX is independent and is incentivized to be commercially successful.

The pricing structure (e.g., a flat account fee per pharmacy connection) PharmX inherited from its utility-like origins survived far longer than the ownership structure that originally justified it.

As CEO Tom Culver put it during its fireside chat at Planet Microcap Conference in June 2026:

"It very much goes to our founding story. So, PharmX was originally developed by market participants essentially to service the industry, rather than, let's say, kind of full-profit enterprise. Yeah, it's very similar, I think, to the kind of the founding story of Mastercard."

I find that comparison useful, because Mastercard's own history is a reminder of what happens when a piece of shared infrastructure that everyone depends on is finally allowed to price its product / service properly: the repricing itself becomes the multi-year growth story, not any change in the underlying network.

None of this happened by accident, and none of it is finished. Management is explicit that simply raising the old flat fee was considered and rejected as the obvious, low-conviction play:

"We could have very simply gone, well, let's just increase our fees for the services that we have today. I think that's the very obvious play. Our view is that it has very short-term and limited impact on our business." (Planet Microcap Conference, June 2026)

Instead, PharmX chose the harder, more durable path: building a genuinely better product, the Marketplace, and repricing around the value that product creates and delivers to pharmacies and suppliers.

IV. The Business Model Transformation: From Flat Fee to Take Rate

The mechanics of the repricing are straightforward, even if the execution is not.

Under the old model, a supplier paid PharmX a fixed monthly fee per pharmacy connection, roughly AUD 17 per connection per month on a blended basis, regardless of how much product actually moved through that connection. It is a model that rewards PharmX for existing, not for helping suppliers sell more.

Under the new model, suppliers pay PharmX a percentage of the GMV, a true take rate, ranging from roughly 0.5% up to 5% depending on the supplier and the category of product being sold.

Two things about how this transition is being executed strike me as unusually thoughtful for a company this size.

First, PharmX is not simply asking existing customers to accept a large, blunt price increase. It is asking them to pay more for something that is demonstrably worth more: a marketplace that helps a supplier win share and automate its own supply chain, rather than a pipe that just moves data back and forth.

As CEO Tom Culver frames it, PharmX is repositioning itself from a technology cost center in a supplier's budget to a growth partner funded out of sales and marketing budget, a much larger and more elastic pool of spend: "We're no longer a technology cost. We're actually now part of their sales and marketing budget and their growth opportunity." (Planet Microcap Conference in June 2026)

Suppliers can also pass a portion of the fee through to the pharmacy, the ultimate buyer, which further eases the conversation around the size of the increase.

Second, and I think this is the more important structural point, PharmX has closed the obvious loophole. In a two-tier pricing world, a sophisticated supplier would be trying to route orders via a low-fee channel and trying to game the system. PharmX has addressed this. Upon contract renewal and signing new suppliers, suppliers no longer get to pick and choose services individually; they contract into the PharmX platform as a whole and pay for the value they draw from it.

The moderator of PharmX's June 2026 Planet Microcap fireside chat summarized it well: "You are unifying the fee structure and making EDI fee equal to Marketplace fee, which makes supplier indifferent between the two. But because the product and customer experience is better with Marketplace, they are

very likely to move to Marketplace, and there is no way to game the system." CEO Tom Culver confirmed that this is an accurate understanding.

V. The Marketplace: A Candid Second Attempt, and the Cold-Start Problem Is Already Solved

PharmX did not get the Marketplace right on the first try.

An earlier version of the Marketplace was piloted in mid-2022 and formally launched in October 2022, built and run largely as a bolt-on rather than a fully integrated product. It generated real interest, and many pharmacies signed up over time, but not real usage. It never crossed from novelty to habit.

Rather than iterate around the edges of a product with a weak foundation, the current management team (e.g., this is not the team who built the first iteration which did not get traction) spent roughly nine months (if not more) building the Marketplace from the ground up on a unified technology stack, fully integrated with two decades of Gateway transaction history, and relaunched it in November 2025 as a true "Single Platform." Because PharmX already knows what a given pharmacy typically orders, the new Marketplace can pre-populate a reorder basket on day one, rather than asking a pharmacist to build a cart from scratch. This is an example of a small-sounding feature / tweak that should help a lot with adoption and removing friction. It addresses the single biggest pain point pharmacists describe: the sheer time cost of ordering.

Since the Marketplace was launched in November 2025, and PharmX reports on a semi-annual basis, we have very limited data about early traction: a bit more than a month in 1H 2026 and full 2H 2026. However, when PharmX provided its 3Q 2026 Update (ended March 31, 2026), it provided a number of data points. Ironically, I have found the full-year 2026 Report less insightful than the 3Q 2026 Update.

Here are several relevant data points showing early traction.

Let's start with GMV.

- April 2026 GMV was over AUD 1.8 million, which was 42% up vs. March 2026 (e.g., March 2026 GMV must have been AUD 1.27 million).
- July 2026 GMV reached AUD 4.7 million. So, July 2026 GMV was up ~3.7x over four months.

There are also several data points regarding the user behavior. PharmX discloses some relevant data points for Target Cohort pharmacies (e.g., those pharmacies that, in the view of PharmX, is a great fit for the Marketplace right now) and for all pharmacies.

- During 3Q 2026, Target Cohort pharmacies had a monthly purchase frequency of 17.4 times. All pharmacies that onboarded had a monthly purchase frequency of 4.5 times in 3Q 2026. In 4Q 2026, these numbers increased to 17.7 and 7.6 times respectively.
- Average monthly spend in 3Q 2026 was AUD 43,000 for the Target Cohort customers. In 4Q 2026, it increased by 71%. Please note that this is a monthly average for the quarter. I assume that the last month was better than the first month, but the company has never disclosed that. Overall, the quality and clarity of operating metrics disclosures leave much to be desired, and the management team would benefit greatly from attending YC Demo Day (DD) and learning what and how to present.

Now, let's discuss registered pharmacies.

- Per the 3Q 2026 Trading Update, registered pharmacies on the Marketplace grew to approximately 1,350 by the end of March 2026.
- I do not believe that a similar metric was disclosed for 4Q 2026 or June 2026 (again, attending a YC DD would be beneficial). Please also keep in mind that "registered" does not mean "actively transacting".

On the cold-start problem, the single hardest thing about launching any two-sided marketplace, PharmX starts from a genuinely unusual position: both sides of the network already exist and are already using the company's infrastructure every day. Seventy suppliers committed to using the Marketplace before it had even gone live. For PharmX, in other words, the question was never whether the two sides of the marketplace would show up. It was whether the product, once built correctly, could convert an already-captive network into habitual, high-frequency usage. The early data above suggests the answer is starting to be yes (at least for the Target Cohort).

VI. Industry Structure: Regulated Fragmentation Creates a Durable Two-Sided Network (Though with Caveats)

A marketplace is only as durable as the fragmentation on both sides of it, and Australian pharmacy is about as durably fragmented as a national industry gets, by design rather than by accident.

On the demand side, Australian regulation restricts pharmacy ownership specifically to prevent the kind of vertical corporatization seen in the United States (CVS, Walgreens) or the United Kingdom (Boots). To own a pharmacy in Australia, an individual must be a licensed pharmacist, and no individual can own more than five or seven pharmacies (it is possible that the limit is determined by a province and that's why I have heard both numbers). The result is a few thousand individuals owning pharmacies. This fragmentation persists even inside the large retail banner groups a casual observer might assume are single corporate entities. Chemist Warehouse, still comprises roughly 500 individually owned pharmacies operating under a shared banner, alongside Priceline (roughly 470), TerryWhite Chemmart (roughly 600), and a long tail of smaller groups and independents. No single owner, and no single banner group, can simply dictate technology or purchasing decisions across the network. That structural fragmentation is exactly what a two-sided marketplace needs in order to be a marketplace, rather than something a handful of large buyers could bypass by negotiating directly with a handful of large suppliers.

On the supply side, there are four major wholesalers, a few major distributors, and few hundred direct suppliers (e.g., manufacturers / brands). PharmX already works with around 200 direct suppliers today; management identifies roughly 300 more as commercially viable additions to the network, leaving real room to add more.

Those suppliers face the mirror-image problem to the pharmacies: reaching a market this fragmented through the industry's traditional distribution model, reps driving between individually owned stores across a country with Australia's geography, is genuinely expensive and inefficient.

In my view, PharmX is building a marketplace for an industry that is not as fragmented as the "dream scenario" every investor wants, but sufficiently fragmented for PharmX to have a good shot at succeeding.

VII. PharmX Faces Favorable Tailwinds

Layered on top of this structural fragmentation are three demand-side tailwinds that I think are underappreciated.

First, an aging population with rising rates of chronic disease is structurally increasing the volume of pharmacy visits and purchases.

Second, the pharmacy retail mix is shifting: management describes an industry that used to split roughly 70% pharmaceutical and 30% retail, front-of-store categories like beauty, wellness, and vitamins, and is now closer to the reverse for many of the major consumer brands selling into pharmacy, a shift that plays directly to the higher-take-rate, higher-margin part of PharmX's opportunity.

Third, the industry is astonishingly inefficient today: management states that roughly half of all pharmacy orders placed through traditional channels arrive unfulfilled, partially fulfilled, or on back order. I am hard-pressed to think of another industry where a merchant places an order for goods and has a coin-flip's chance of actually receiving what they ordered on time. That is not a subtle inefficiency to arbitrage away. It is a fundamental defect in how the industry currently operates, and it is exactly the kind of defect that a real-time, integrated marketplace is built to fix.

VIII. The Sigma Alliance: From Existential Threat to Anchor Tenant

The single most important recent event in the PharmX story, and the one I think the market has least understood, is the company's alliance with Sigma Healthcare and Chemist Warehouse.

Some background: Sigma Healthcare, one of Australia's largest pharmaceutical wholesalers, completed a merger with Chemist Warehouse, the country's dominant pharmacy retail brand (banner group), creating a combined entity with enormous leverage over the industry PharmX serves. If someone could build an alternative solution, solve the cold start problem, and persuade others to join the new platform, it would have been Sigma. That is about as close to an existential threat as a company PharmX's size can face. The alliance that resulted was finalized in February 2026.

- PharmX issued Sigma roughly 60 million new shares, on the order of 10% of the company's pro forma fully diluted share count, at AUD 0.145 per share.
- Then PharmX took the entire proceeds and paid them to Sigma as an establishment fee (approximately AUD 8.7 million).

If Sigma hits certain milestones (which have not been disclosed), Sigma could get another 10% and bring its ownership stake to 20%.

In a way, one can say that PharmX is paying the price to convert an existential threat into a long-duration, deeply aligned commercial partner.

IX. Key Actors: Management Team, BoD, Large Shareholders

CEO Tom Culver joined PharmX in 2023 and has been the architect of the current strategy, the Marketplace rebuild, and the take-rate transition. His prior operating experience includes founding Goodments, an app aimed at making investing more accessible to younger, first-time investors, which was later acquired by the ASX-listed Douugh Ltd, where Culver subsequently served as COO through 2023. Douugh's own share price performance over that period was weak, and I would not want to wave that away.

Non-executive Chairman Nick England brings a genuinely relevant background: a UK-trained pharmacist by trade, with senior roles at major pharmaceutical wholesalers Unichem and AAH earlier in his career. England's involvement with PharmX (then Corum) began with a 2019 capital raise in which he and co-investors took control of the board and initiated the strategic overhaul that led, eventually, to today's business.

I would flag one related-party item / transaction. Chairman England is also a director of Alchemy Healthcare, the counterparty from which PharmX originally licensed the intellectual property underlying the Marketplace, under a revenue-share arrangement that PharmX terminated early in December 2024 in order to take full, unencumbered ownership of that IP, a resolution that removes rather than extends the related-party exposure.

Dennis Bastas and/or related entities own roughly a 10% stake and have Board representation. Dennis Bastas is the founder of Arrotex Pharmaceuticals and the executive chairman of DBG Health. He has a wide range of interests and a strong presence in Australia's pharmaceutical industry.

X. Capital Structure At a Glance

Here is PharmX's capital structure at a glance, as of September 8, 2026.

Last reported period, fiscal		2026
Share price	A\$	0.090
Actual SO	# M	666.1
Dilution - Performance Rights	# M	26.5
F/D S/O	# M	692.6
Market cap	A\$ M	62.3
Debt	A\$ M	-
Leases	A\$ M	0.5
Debt & DE	A\$ M	0.5
Cash and CE	A\$ M	2.57
EV	A\$ M	60.3

XI. Valuation Is Highly Attractive ... *IF* PharmX Executes

As I alluded to earlier, PharmX is not statistically cheap on trailing numbers.

But if management executes and successfully transitions the company to a take rate based revenue model, those trailing numbers would be barely relevant.

Here is the core napkin math (some of it was covered in the introduction), using PharmX's own FY2026 GMV base of AUD 26 billion. At a 2% blended take rate applied to just 10% of total GMV migrating onto take-rate pricing, PharmX would generate approximately AUD 52 million of revenue, roughly 6x its FY2026 revenue base. At a 50% EBITDA margin, that would be approximately AUD 26 million of EBITDA.

The tables below show how sensitive that outcome is to the two variables that matter: take rate and share of GMV migrated (holding the 50% EBITDA margin assumption constant) (numbers are in millions of AUD).

Potential Revenue Opportunity: TR (horizontal) vs. Share of GMV Migrated (vertical)

	10%	15%	20%	25%	30%	35%	40%	45%	50%
0.5%	13	20	26	33	39	46	52	59	65
1.0%	26	39	52	65	78	91	104	117	130
1.5%	39	59	78	98	117	137	156	176	195
2.0%	52	78	104	130	156	182	208	234	260
2.5%	65	98	130	163	195	228	260	293	325
3.0%	78	117	156	195	234	273	312	351	390

Potential EBITDA Opportunity: TR (horizontal) vs. Share of GMV Migrated (vertical)

	10%	15%	20%	25%	30%	35%	40%	45%	50%
0.5%	7	10	13	16	20	23	26	29	33
1.0%	13	20	26	33	39	46	52	59	65
1.5%	20	29	39	49	59	68	78	88	98
2.0%	26	39	52	65	78	91	104	117	130
2.5%	33	49	65	81	98	114	130	146	163
3.0%	39	59	78	98	117	137	156	176	195

Many of the cells in that table imply an EBITDA figure larger than PharmX's entire current enterprise value. Does it feel a bit surreal? Yes, I have that feeling. But I also believe that some of the outcomes (maybe even all outcomes) in those tables are achievable with strong execution.

But management's own math, laid out publicly at the June 2026 Planet Microcap Conference, points in the same direction from a different angle: of the AUD 23 billion of total GMV (e.g., CEO was using FY 2025 GMV), management believes roughly AUD 10 billion is a reasonable medium-term opportunity to capture

through the Marketplace, and at a 2% average take rate on that AUD 10 billion, the company would generate approximately AUD 200 million of revenue; at or above scaled marketplace EBITDA margins of around 50%, with limited ongoing capital expenditure once the platform is built, EBITDA could reach AUD 100 million.

I have deliberately left out the timeline, which is uncertain. It may take many years to get to these outcomes. When PharmX reported its full 2026 results, it indicated that it expected to reach a run-rate of AUD 100 million of GMV exiting FY 2027 (ending June 30, 2027). This data point shows that investors should not be expecting an overnight transformation.

XII. Risks

I see several real risks and open questions here.

1. Execution risk. The entire thesis is a bet that a small management team can simultaneously convert a legacy customer base to new pricing, scale a rebuilt marketplace product, and manage an increasingly complex strategic alliance, all without diluting shareholders.
2. Wholesaler concentration. There are four large wholesalers in Australia. Working with them presents strategic challenges that PharmX's management team needs to carefully navigate.
3. Take-rate acceptance by wholesalers and brands / manufacturers.
4. Disclosure opacity. PharmX does not currently break out Marketplace-specific revenue or profitability as a separate reporting segment, citing a policy of not disclosing segments below a 10% revenue threshold, which limits how precisely an outside investor can track the pace of the transition between quarterly updates and management commentary. As mentioned, key operating metrics disclosures are inconsistent over time.

XIII. Core Qualitative Assumptions

If I were asked to summarize the bull case in terms of the core qualitative assumptions ("What I Have to Believe" summary), an investor in PharmX today must believe that:

1. PharmX's network position, effectively the entire Australian and New Zealand pharmacy industry on one side and effectively all of the major wholesalers and suppliers on the other, is durable, and that the fragmentation on both sides, which is regulatory in origin rather than incidental, is sufficient to build and maintain a true two-sided marketplace.
2. Management can continue converting that network from a flat-fee connectivity utility into a genuine take-rate marketplace, on a timeline of years rather than quarters, without losing significant customers along the way in the process.
3. Major wholesalers would stay on the platform (or at least 3 out of 4; 4 is ideal; 3 is doable; 2 is risky; 1 would make success of the marketplace questionable).

If those three things are true, the napkin math largely takes care of itself over time. If they are not, this is an expensive small-cap technology company trading on a hopeful narrative (though its technology, infrastructure, and network should have value and may attract an acquirer).

XIV. Catalysts

1. Continued Marketplace GMV growth.
2. Onboarding of a second, third, and fourth major wholesaler onto the Marketplace, which should act as a big unlock for the next leg of pharmacy adoption.

XV. Disclaimer

The analysis presented is an opinion of the author. The author and/or affiliated entities are long PharmX Technologies (PHX AU) shares. Various factors may influence the analysis or the opinions expressed. The author does not assume any obligation to update the analysis. The author and/or affiliated entities may sell, buy, or execute any other transactions with PharmX Technologies shares and/or related and/or derivative securities at any time.

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Catalyst

1. Continued Marketplace GMV growth.
2. Onboarding of a second, third, and fourth major wholesaler onto the Marketplace, which should act as a big unlock for the next leg of pharmacy adoption.

Messages

Subject	Questions
Entry	09/09/2026 04:32 PM
Member	bafana901

AI tells me that 85% of the supply moves through the 4 wholesale groups. Pharma wholesalers run on razor thin margins and I dont think they will be in a position to give up 2% of the margin. Further, a 2% margin would incentivize the wholesalers to build ordering systems. I cant imagine that this can be so difficult.

This is my superficial knee jerk response. Look forward to your views.

Subject	Testimonial
Entry	09/09/2026 11:09 PM
Member	roojoo

Have you had the chance to speak to a supplier in the target market who can confirm the usefulness of their marketplace? If one of them says a 0.5% take rate pays for itself and their business has grown since switching to marketplace, that would help the PharmX salesforce and would help potential investors build confidence.